

Functional Maintenance & Prevention Documentation

What to write · When to use it · How to defend it · SNF Therapy · 2026

Jimmo v. Sebelius (2013) — **Know This:** Medicare CANNOT deny coverage solely because a patient is not improving or has reached a plateau. Coverage is required when skilled care is needed to MAINTAIN current function OR PREVENT / SLOW anticipated decline — even with no expectation of improvement. Your documentation must prove the SKILLED NEED, not just the outcome.

Every Maintenance Note Must Answer These 4 Questions

1	What function is being maintained? Name the specific skill: transfer safety, safe ambulation, swallowing function, ADL independence, etc.
2	What decline would occur without skilled care? Be specific: increased fall risk, aspiration risk, loss of transfer independence, caregiver injury risk.
3	Why can't a non-skilled person do this? Explain why restorative staff or family cannot safely perform or monitor this program without therapist.
4	What skilled act did the therapist perform today? Document the modification, clinical judgment, or assessment made this session — this IS the skilled service.

Opening / Justification Statements

GENERAL — Use to open any maintenance note

"Skilled [PT/OT/ST] is required to maintain patient's current functional level. Without ongoing skilled intervention, clinical judgment supports that patient would experience decline in [transfer safety / ambulation / swallowing / ADL performance] secondary to [diagnosis / progressive condition / deconditioning risk]."

PLATEAU — Patient not progressing but still needs skilled care

"Patient has reached a functional plateau with respect to [skill] however continued skilled [PT/OT] is medically necessary to maintain current level of function and prevent the anticipated decline that would occur without skilled oversight and program modification."

TRANSITION — Moving from restorative to maintenance

"Patient has achieved maximum therapeutic benefit and has been transitioned to a skilled maintenance program. Continued skilled oversight is required to ensure program integrity, monitor for regression, and modify the plan of care as the patient's condition evolves."

Prevention of Decline Language

FALL RISK

"Without continued skilled PT intervention, patient is at high risk for falls based on [history of falls / vestibular impairment / lower extremity weakness]. Skilled care is necessary to monitor for early signs of decline, modify the exercise program, and reinforce safe movement strategies that cannot be safely delegated to non-skilled staff."

ASPIRATION / SWALLOWING RISK

"Skilled SLP is medically necessary to monitor patient's swallowing function and prevent aspiration events. Patient's [diagnosis] places them at ongoing risk for deterioration in swallowing safety. Without skilled monitoring and technique reinforcement, patient would be at increased risk for aspiration pneumonia and a higher level of care."

ADL / CAREGIVER SAFETY

"Skilled OT is required to maintain patient's current level of assisted [dressing / transfers / bathing] and to prevent decline that would increase caregiver burden and injury risk. Non-skilled staff cannot safely assess or modify the assist techniques required for this patient."

Diagnosis-Specific Language

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Parkinson's Disease "Patient's diagnosis of Parkinson's disease is progressive in nature. Skilled PT/OT is required on an ongoing basis to monitor for functional changes, modify the exercise program to address evolving motor symptoms, and prevent the accelerated functional decline associated with inactivity in this population."	CVA / Stroke "Patient presents with residual deficits secondary to CVA including [hemiplegia / aphasia / dysphagia]. Skilled therapy is necessary to maintain current functional gains, monitor for regression, and modify the treatment program based on ongoing neurological recovery — clinical judgment required throughout."
Dementia / Cognitive Impairment "Patient's cognitive impairment requires ongoing skilled OT to monitor for functional decline, modify environmental adaptations, and provide caregiver training as the patient's presentation evolves. Non-skilled staff cannot safely assess cognitive changes or adjust the program accordingly."	CHF / Cardiac Conditions "Patient's cardiac condition requires skilled PT monitoring during exercise to assess for signs of cardiac compromise and modify the program based on vital sign response and symptom changes. This level of clinical monitoring cannot be safely performed by restorative or non-skilled staff."
COPD / Pulmonary "Skilled PT is required to monitor oxygen saturation and respiratory response during functional mobility and to adjust activity intensity based on clinical findings. Patient's pulmonary status places them at risk for rapid decompensation without skilled oversight during therapy."	Orthopedic / Post-Surgical "Patient continues to require skilled PT to maintain functional gains post-[procedure] and to monitor for complications including [DVT signs / wound changes / weight-bearing tolerance decline]. Without skilled oversight, patient is at risk for regression and potential need for re-hospitalization."

Why a Non-Skilled Person Cannot Perform This Program

“	Patient's program requires ongoing clinical modification secondary to [fluctuating cognition / pain levels / postural instability / vital sign changes] — a restorative aide cannot safely monitor or adjust these parameters without direct therapist oversight.
“	Skilled therapist judgment is required at each session to assess patient's response to the maintenance program and modify activity parameters, equipment, or technique. This cannot be delegated to non-skilled staff without risk to patient safety and program effectiveness.
“	The complexity of patient's presentation — including [multiple comorbidities / medication changes / recent acute illness] — requires a licensed therapist to continuously reassess and adapt the program. A restorative program alone is insufficient to address this level of clinical complexity.
“	Patient requires skilled instruction and ongoing feedback to maintain safe technique during [transfers / ambulation / swallowing]. Non-skilled reinforcement alone has been insufficient to maintain safe performance, as evidenced by [near-fall / increased assist level / technique breakdown observed].

What to NEVER Write in a Maintenance Note

"Patient on maintenance program — no skilled need at this time."	This is an automatic denial. Skilled need IS the maintenance oversight — document it.
"Patient at plateau — will discharge to restorative."	Only appropriate if patient truly no longer needs skilled monitoring. If they do, document why.
"No changes this visit."	Every visit must document a skilled act — an assessment, modification, or clinical judgment made.
"Caregiver/aide can perform this program independently."	If true, skilled therapy is not justified. Only write this at the time of actual discharge.

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"Patient not progressing — continuing per physician order."	Physician order alone does not justify coverage. You must document the skilled need each session.
Before signing a maintenance note, confirm: ✓ Named the functional skill being maintained ✓ Documented what decline would occur without skilled care ✓ Explained why non-skilled staff cannot perform this program ✓ Documented the skilled act performed today ✓ Linked to the patient's diagnosis and clinical presentation	